MEDICAL ICU PROGRESS NOTE - HOSPITAL DAY 3
SYNTHETIC NOTE FOR SOFTWARE TESTING. NOT A REAL PATIENT.

Patient: 61-year-old female
Admitting service: Medical ICU
Allergies: Sulfa (hives)
Reason for ICU admission: Hemorrhagic shock from upper GI bleed (duodenal ulcer), complicated by aspiration pneumonia and acute kidney injury.

History of present illness:
Presented on day 1 with hematemesis and melena. On arrival HR 128, MAP 52. Intubated for airway protection before endoscopy. EGD on day 1 showed a 1.5 cm duodenal ulcer with a visible vessel, treated with clips and epinephrine injection. Octreotide gtt was started in the ED for possible varices and was stopped on day 1 after EGD showed no varices. Developed fever and a right lower lobe infiltrate on day 2, treated as aspiration pneumonia.

Interval events (last 24 h):
No further hematemesis. Received 2 units PRBC overnight (4 units total since admission). Remains intubated. NE requirement down from 0.15 to 0.08 mcg/kg/min.

Past medical history: Hypertension, osteoarthritis (daily ibuprofen use), GERD.
Home medications: Metoprolol 25 mg BID, ibuprofen 400 mg TID, omeprazole 20 mg daily.

CURRENT MEDICATIONS
- NE gtt 0.08 mcg/kg/min, titrate to MAP >= 65
- PPI gtt (pantoprazole) 8 mg/hr for 72 h, then IV BID
- Propofol 30 mcg/kg/min for sedation, RASS goal -1 to -2
- Fentanyl 25 mcg/hr for analgesia
- Pip-tazo 4.5 g IV q8h for aspiration pneumonia (started day 2)
- Vanc discontinued on day 2 after MRSA nares PCR negative
- Octreotide stopped day 1 (see HPI)
- Enoxaparin DVT prophylaxis HELD due to active GI bleed; SCDs in place
- Home metoprolol held for hypotension
- Ibuprofen stopped permanently (ulcer)
- Consider IV iron once bleeding has resolved

LABS (this morning)
Hgb 68 g/L (low; was 61 g/L before transfusion)
Plt 145 x10^9/L (Day 1: 92 x10^9/L)
WBC 11.8 x10^9/L
INR 1.9 (elevated)
Na 139 mmol/L (normal)
K 4.2 mmol/L (within normal limits)
HCO3 19 mmol/L
Urea 14.2 mmol/L (elevated)
Cr 212 umol/L (AKI; baseline 70 umol/L)
Lactate 4.1 mmol/L (elevated, down from 6.3)
Glucose 7.1 mmol/L (at target)
Albumin 24 g/L (low)
Mg 0.8 mmol/L

DEVICES
Endotracheal tube, right internal jugular central line, radial arterial line, orogastric tube, Foley catheter.

ASSESSMENT AND PLAN
1. Hemorrhagic shock from duodenal ulcer bleed: hemodynamics improving. Continue PPI gtt. Transfuse for Hgb < 70 g/L. Repeat Hgb q6h. GI to re-scope if rebleeding.
2. Aspiration pneumonia: continue pip-tazo, follow sputum culture. Vanc off.
3. AKI, likely prerenal/ischemic: avoid nephrotoxins, renally dose medications, strict I/O. Nephrology aware.
4. Respiratory failure: daily spontaneous breathing trial once off pressors.
5. Ibuprofen stopped permanently; H. pylori serology sent.

Code status: Full code.
Family: Husband updated at bedside.